

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Section 1.3 — Inside AISH: The Application, the Spend-Down, the Wait, the Appeal You No Longer Have

Most workers do not arrive at AISH having read the program rules. Most workers arrive at AISH because WCB ended, or because the disability did not arise at work and was therefore never a WCB claim, or because Canada Pension Plan Disability has a contribution-history requirement they cannot meet, or because Employment Insurance sickness benefits have run out. Most workers arrive at AISH late, with their finances already substantially depleted, having relied on family supports and credit and the assumption that the next program would catch them. AISH is the program of last resort. It is administered as if applicants are gaming the system, not as if they have arrived after every other system has failed.

This section walks the AISH application in operational detail. It is a long section because the application is a long process and the operational reality of that process is the operational reality of the system the document is naming.

What you must establish

To qualify for the Assured Income for the Severely Handicapped program, an applicant must establish three things. First, that they have a permanent disability — defined under the Assured Income for the Severely Handicapped Act as a medical condition that is likely to remain with the applicant for the rest of their life and that substantially limits their ability to earn a livelihood. (*Source: Assured Income for the Severely Handicapped Act, R.S.A. 2000, c. A-45.1, section 3; Assured Income for the Severely Handicapped Regulation, Alta Reg 91/2007.*) Second, that they meet the financial-eligibility tests, which include both an income test and an asset test. Third, that they are an Alberta resident, eighteen or older, and not in a federal correctional facility serving a sentence of two years or more.

The medical determination is made on the strength of the Part B Medical Report, completed by the applicant's physician and submitted to the Board's adjudication staff. The financial determination is made on the strength of the Part A application, completed by the applicant, including disclosure of all bank accounts, all investment accounts, all real estate holdings, all vehicles, all life insurance with cash value, all employment income, all government benefit income from any other source, and all support payments received or paid. The Government of Alberta will request bank statements going back twelve months. (*Source: Government of Alberta — How to Apply for AISH, 2026; AISH Policy Manual.*)

The medical evidence problem

The Part B Medical Report requires a physician signature. Specifically, it requires the signature of a "regulated medical professional" whose qualifications are defined in regulation. For most components of the form, that means a physician — a medical doctor, either a general practitioner or a specialist with treating relationship to the applicant. Nurse practitioners are permitted to complete certain components of the Part B but, as of the 2026 program design, are not authorized to sign all sections of the form, despite NP scope of practice having expanded significantly under recent provincial reforms. (*Source: Edmonton Journal — "I can't sign the medical: Alberta nurse practitioner says AISH rules present barrier for patients," January 11, 2024; Government of Alberta — AISH application guidelines, 2026.*)

This requirement collides directly with the Alberta primary-care environment. As of 2024, six hundred and fifty thousand Albertans had no family physician. (*Source: Alberta Medical Association — AB's Family*

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Medicine Crisis statement, 2024; CBC News — "Concerns grow as more and more Albertans can't find a family doctor," April 27, 2022; ThinkHQ survey commissioned by AMA, January 2024.) The number of family practices accepting new patients in Alberta dropped from eight hundred and eighty-seven in 2020 to one hundred and sixty-four in 2024. (Source: *Medscape Medical News — "Alberta's Family Doctor Shortage Involves More Than Numbers," October 6, 2025.*) Sixty-one per cent of Alberta family physicians, surveyed by the AMA in 2024, were considering leaving the provincial healthcare system. (Source: *Global News — "61% of family physicians in Alberta considering leaving provincial health-care system: Report," January 24, 2024.*)

The applicant who has no family physician must obtain one to complete the AISH Part B. This is not a quick task. Wait times for new patient acceptance, where any spots exist at all, may be six to twenty-four months. The applicant may pay out of pocket for a private clinic appointment to obtain a Part B signature, but private clinics are not always willing to complete government forms for patients without an established relationship. Walk-in clinics generally will not. The applicant may seek the signature of a physician with whom they have only a brief treating relationship — an emergency room physician, an urgent-care physician, a specialist seen once for a referral. Such physicians frequently decline to complete the Part B because they have not seen the applicant longitudinally and cannot attest to permanence. The Part B requires attestation to permanence. The applicant whose disability is real and permanent and who cannot find a physician to attest to it is, for the purpose of the program, ineligible.

The Government of Alberta covers a one-time supplemental payment of one hundred and fifty dollars to compensate the physician for completing the Part B. (Source: *Government of Alberta — "How to apply for AISH" — note: "your doctor may charge you for this service," 2026.*) Current physician fees for completing complex disability forms range from two hundred and fifty to four hundred dollars, sometimes higher for specialist-level forms. (Source: *Alberta Medical Association — Schedule of Medical Benefits, uninsured services, 2025.*) The supplemental payment does not cover the actual cost. The difference is paid out of pocket by the applicant — at the moment in their life when out-of-pocket capacity is at its lowest.

The asset spend-down

The AISH financial-eligibility tests cap liquid assets at five thousand dollars for a single applicant and ten thousand dollars for a household with dependents. (Source: *Assured Income for the Severely Handicapped Regulation, Alta Reg 91/2007, sections 4-7; Government of Alberta — AISH financial eligibility, 2026.*) Liquid assets are defined broadly. The category includes:

- Chequing and savings accounts at any bank, credit union, or other financial institution
- Tax-Free Savings Accounts (TFSAs)
- Registered Retirement Savings Plans (RRSPs)
- Registered Retirement Income Funds (RRIFs)
- Non-registered investment accounts
- Guaranteed Investment Certificates (GICs) and term deposits
- Cash on hand
- The cash surrender value of life insurance policies
- Stocks, bonds, mutual funds, ETFs, and similar securities

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- Cryptocurrency holdings
- Accounts receivable, including loans owed to the applicant by family or third parties

A primary residence is exempt from the asset cap, though the equity in the residence is reviewed in some circumstances. A vehicle is exempt up to a value the Board determines. Household furnishings, personal effects, tools required for self-employment, and certain other categories are exempt. A Registered Disability Savings Plan (RDSP) is excluded from the asset test up to defined federal thresholds. Almost everything else must be liquidated.

The operational reality of this is what the working-age reader most needs to understand. An applicant with twenty-three thousand dollars in an RRSP, accumulated across two decades of disciplined retirement saving, must collapse the RRSP. Collapsing an RRSP triggers withholding tax — ten per cent on amounts up to five thousand dollars, twenty per cent on amounts between five thousand and fifteen thousand, thirty per cent on amounts above fifteen thousand. (*Source: Canada Revenue Agency — RRSP withholding tax rates, 2026.*) The withheld amount goes to the Canada Revenue Agency. The remainder is added to the applicant's income for the year and taxed at the applicant's marginal rate at year-end. For an applicant who had any employment income earlier in the year before the disability, or who collapsed multiple registered accounts, the year-end tax bill can be substantial.

After tax, the applicant is left with a fraction of the saved amount. This is the amount the AISH cap requires them to spend down. Once they are below the cap, AISH will pay one thousand nine hundred and forty dollars per month. The retirement savings are gone, the tax has been paid, the runway has been consumed, and the program — the program — pays subsistence.

A TFSA must be liquidated. There is no withholding tax on TFSA withdrawals, but the TFSA contribution room is permanently consumed for the year of withdrawal in the case of a TFSA that has been overcontributed; there are technical rules around recontribution that the applicant typically does not understand and does not have time to learn. The vehicle worth more than the exemption must be sold. The applicant who lives in a region of Alberta without functional public transit then has no transportation. The applicant who needed the vehicle to attend medical appointments now requires medical transportation services that AISH does not fully cover.

The home that is exempt and not exempt

The primary residence is exempt from the asset cap. This is sometimes presented in public-facing AISH summaries as if it means the applicant who owns a home is in a stable position. It does not mean that.

The home is exempt from being counted as a liquid asset. The home is not exempt from carrying costs. Property taxes continue. Property insurance continues. Utility bills continue. Maintenance, repairs, and replacement of failing systems — furnaces, roofs, water heaters, foundations — continues. Strata fees, condominium fees, and homeowners' association fees continue. None of these costs are reimbursed by AISH. All of them are paid out of the one thousand nine hundred and forty dollars per month.

The applicant who owns a paid-off home in a moderate-cost region of Alberta may find that property tax alone consumes one hundred and fifty to four hundred dollars per month, depending on assessed value and municipality. Insurance adds another seventy-five to one hundred and fifty. Heating, electricity, water, and waste add three to five hundred per month depending on season. Maintenance averaged across the year —

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even at minimum — adds another hundred. Before food, before medications, before clothing, before any other expense, the homeowner on AISH may have spent six to twelve hundred dollars per month on home carrying costs alone. The remaining seven to thirteen hundred is what is left for everything else.

The applicant who falls behind on property taxes or insurance can lose the home to municipal tax sale or to insurance lapse. This is documented in housing-counselling literature in Alberta and across Canadian provinces with similar program structures. (Source: *Canadian Centre for Policy Alternatives — "Disability and Housing Insecurity in Canada," 2023; Alberta Council of Disability Services — annual policy briefs, 2022-2025.*) The "exempt" home becomes the asset the applicant loses last, but loses nonetheless, when the program income proves inadequate to maintain it.

The wait

Once the application is submitted with complete medical and financial documentation, the wait begins. The Government of Alberta has not, as a matter of public policy, published current AISH application processing times. Disability advocacy organizations and applicants have documented wait times of six months to over two years, with significant variation by region and adjudication office. (Source: *Inclusion Alberta ADAP Fact Sheet, December 2025; AUPE — AISH Caseworker Caseload Statement, 2025.*)

During the wait, the applicant has no AISH income. If the applicant has exhausted Employment Insurance sickness benefits, terminated WCB benefits, and depleted personal savings to the AISH asset cap in anticipation of approval, the applicant has no income at all during the wait. Some applicants are eligible for Income Support (the general welfare program in Alberta), at substantially lower rates than AISH, while waiting. Income Support is itself asset-tested and income-tested. The applicant's family may step in. The applicant may use food banks. The applicant may run out of medications. The applicant may experience a mental-health crisis, hospitalization, eviction, or homelessness, while waiting for the program designed to support them.

This wait is not a glitch. It is the operational reality. It is documented across multiple advocacy reports and across the disability community's own organizational publications. (Source: *Inclusion Alberta — "Experiences with Family Support for Children with Disabilities," January 2025; Inclusion Alberta / ALIGN — "PDD Current State: Critical and Urgent," January 2025; Disability Action Hall — public statements 2024-2026.*)

The 2024 caseworker elimination

In 2024-25, the Government of Alberta eliminated individual AISH caseworkers province-wide and replaced them with a centralized call centre. (Source: *AUPE — AISH Caseworker Caseload Statement, 2025; Inclusion Alberta ADAP Fact Sheet, December 2025.*) Prior to this change, an AISH recipient had a named caseworker who was familiar with the file and reachable by direct phone or email. After the change, the recipient calls a general number, waits on hold, speaks to a different agent on each call, and may need to re-explain their situation each time. Caseworker caseloads, prior to elimination, had reached up to two hundred files per worker — more than double the recommended maximum of ninety to one hundred and five. (Source: *AUPE — AISH Caseworker Caseload Statement, 2025.*)

The call centre model is documented as inaccessible to recipients with social anxiety, agoraphobia, severe communication-related disabilities, and brain injuries — categories of disability that are themselves

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AISH-qualifying. (Source: *Inclusion Alberta ADAP Fact Sheet, December 2025*.) The applicant who qualifies for AISH on the basis of a condition that makes phone calls difficult must, to access the program, navigate a phone-only contact system. The system was not designed around its users. It was designed around administrative cost containment.

The appeal you used to have

Until December 2025, an AISH applicant or recipient who disagreed with a Board decision had access to an independent Citizens Appeal Panel. The Panel was a quasi-judicial body composed of community members with relevant expertise, operating at arm's length from the Government and from the AISH adjudication staff. The Panel's decisions were binding on the program. Applicants who succeeded at the Panel had their decisions reversed and benefits reinstated, often with retroactive payment.

On December 9, 2025, the Alberta Legislature passed Bill 12, the *Disability Assistance Programs Act*. (Source: *Government of Alberta — Bill 12, Disability Assistance Programs Act, Royal Assent December 9, 2025*.) Bill 12 abolished the independent Citizens Appeal Panel. In its place, Bill 12 established a Medical Review Panel composed of medical professionals selected by the Government. The Medical Review Panel reviews disputed eligibility decisions. Its decisions are final. There is no independent appeal from a Medical Review Panel decision. Judicial review remains available only on procedural grounds, not on the merits.

Inclusion Alberta's November 26, 2025 statement on Bill 12 stated that the legislation "erodes rights and deepens poverty for Albertans with disabilities." (Source: *Inclusion Alberta — Media Release, November 26, 2025*.) The Alberta Medical Association, the Alberta Union of Provincial Employees, Disability Action Hall, and the People's Alliance for Disabled Albertans, among others, opposed Bill 12. No major Alberta disability advocacy organization publicly endorsed it. The municipal coalition documented in the Alberta Disability System Breakdown Coalition and UN CRPD Addendum — including the cities of Edmonton, Calgary, Lethbridge, Camrose, Claresholm, Red Deer, and Medicine Hat — has called for the AISH-to-ADAP transition to be paused pending meaningful consultation, in part because of the appeal-rights removal Bill 12 effected.

The applicant who, as of 2026, is denied AISH or has benefits terminated does not have the remedy that applicants had as recently as one year ago. Canada ratified the United Nations Convention on the Rights of Persons with Disabilities in 2010. (Source: *United Nations General Assembly Resolution A/RES/61/106, 13 December 2006; Canada — instrument of ratification deposited March 11, 2010*.) Article 13 of the Convention protects access to justice for persons with disabilities, including procedural accommodations and effective remedy. Bill 12 has, in operational effect, removed the effective remedy for AISH eligibility disputes in Alberta. The campaign's Coalition and UN CRPD Addendum documents the engagement of Articles 12, 13, 19, and 28 by the Bill 12 changes; that documentation is incorporated into this Section by reference.

What the applicant has by the end

The applicant who completes the AISH process — finds the physician, completes Part B, completes Part A, liquidates assets, waits the wait, receives approval — has by the end of the process a benefit of one thousand nine hundred and forty dollars per month, an asset cap they must remain under for as long as they receive the benefit, a cohabitation calculation that ties their benefit to their spouse's income, a call centre as their primary point of contact with the program, and no independent appeal if the program's

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adjudicators err.

The applicant has also, in most cases, depleted retirement savings, personal savings, and family financial supports during the application and approval process. The runway is gone. The benefit is the floor. From this point forward, until age sixty-five, the applicant is inside AISH. Section 1.4 walks the AISH-to-ADAP transition that begins July 1, 2026 — the next reassessment, the next reduction, the next cliff that a successful AISH applicant in 2025 had no way of anticipating when they applied.